

ADRABETADEX Document A

MLR briefing on launch background, core assumptions, commercial payer treatment, government pricing, and provider economics

Prepared for client discussion. Content is grounded in the supplied source pack and current CMS, HRSA, Medicaid, Medicare, and public payer materials reviewed on March 30, 2026.

Executive conclusion

ADRABETADEX enters U.S. launch planning as a clinician-administered, high-cost therapy for infantile-onset NPC with a priority-review PDUFA date of August 17, 2026 in the supplied research pack. The commercialization deck anchors launch on medical-benefit buy-and-bill, premium pricing acceptance, and a 50% commercial / 50% Medicaid payer mix.

The source pack supports a definite access conclusion: commercial and Medicaid coverage can be obtained, but neither channel behaves like friction-free reimbursement. Public payer precedent in NPC and related neurologic rare diseases shows structured prior authorization, combination-therapy controls, continuation criteria, site-of-care management, and, in some cases, specialty-pharmacy channel enforcement.

For MLR review, the core issue is not whether the product is clinically meaningful. The core issue is whether the launch narrative uses reimbursement language that matches real payer and provider mechanics. The attached materials support a disciplined claim set built on documented coding, Medicaid, 340B, and provider-cash-flow realities.

Background and core assumptions carried into launch planning

- The internal commercialization deck sets premium pricing acceptance at up to **\$1.8 million** and uses a base-case annual price of **\$1.1 million** with a **15%** gross-to-net discount.
- The same deck models a **50% commercial / 50% Medicaid** payer mix and treats the channel as medical benefit buy-and-bill.
- The deck uses a launch base case of **175 currently diagnosed** infantile patients with **80%** peak penetration, **60 EAP** patients converting at launch, **250** undiagnosed patients with **65%** peak penetration, **20** misdiagnosed patients with **45%** peak penetration, and **25** newly born patients with **75%** peak penetration.
- The provider concept brief references a working launch WAC of **\$39,000 per 900 mg vial every two weeks**, which equates to an annual drug acquisition exposure of **\$1,014,000** before provider overhead.
- The product launches before a permanent product-specific HCPCS code is guaranteed to be live. The research pack therefore treats unclassified-code billing, NDC capture, and documentation quality as day-one operational issues, not later clean-up items.

Commercial payer perspective and treatment of analogous therapies

Analogue or policy	Current public management pattern	Direct relevance for ADRABETADEX
UHC Miplyffa (effective 4/1/2025)	Requires NPC diagnosis, treatment of neurologic manifestations, use with miglustat, no concurrent Aqneursa, and positive clinical response for reauthorization.	Commercial plans already use combination restrictions and continuation rules in NPC. ADRABETADEX will face the same demand for documented ongoing benefit.

Analog or policy	Current public management pattern	Direct relevance for ADRABETADEX
UHC Aqneursa (effective 4/1/2025)	Requires NPC diagnosis, treatment of neurologic manifestations, no concurrent Miplyffa, and positive clinical response for renewal.	Commercial plans already exclude overlapping high-cost NPC regimens. ADRABETADEX does not launch into an unrestricted combination environment.
Aetna Spinraza	Uses baseline assessment, specialist involvement, and continuation review in a high-cost intrathecal rare disease therapy.	Payers already treat intrathecal neurologic products as utilization-managed medical-benefit drugs, not as automatic pass-throughs.
Aetna Brineura	Ties use to provider expertise and functional outcome monitoring in an intraventricular pediatric disease setting.	Route complexity and center capability are already accepted reasons for payer management in pediatric neurodegeneration.

The commercial takeaway is definite: ADRABETADEX will not be treated as a blank-check medical-benefit product. Payers will require a tight indication definition, center-level documentation discipline, and a continuation endpoint that is auditable in children who may not be able to demonstrate classic functional improvement.

340B pricing, Medicaid behavior, and dual-eligible implications

340B pricing is a statutory acquisition formula, not a discretionary rebate. HRSA states that the 340B ceiling price equals **AMP minus URA**. For a new drug before AMP is available, HRSA requires manufacturers to estimate the ceiling price from **WAC minus the applicable rebate percentage**.

Using the source-pack WAC of **\$39,000** per 900 mg vial, the launch-era 340B acquisition estimate is **\$29,991** per vial under the standard innovator rebate percentage of 23.1%. If the product later qualifies for the exclusively pediatric 17.1% basic rebate category, the comparable estimate is **\$32,331** per vial.

The Medicaid channel is a coverage tailwind and an operational burden at the same time. EPSDT requires states to furnish medically necessary services for beneficiaries under 21, including services needed to correct or ameliorate a condition even when the service is not otherwise listed in the state plan. At the same time, Medicaid PAD rules tie payment availability and federal matching funds to utilization-data and NDC capture requirements for physician-administered drugs.

That combination means ADRABETADEX will have pediatric Medicaid access, but payment will only convert to cash when the claim carries complete NDC data, correct units, and clean documentation. For launch operations, Medicaid prior authorization lead time, NDC formatting accuracy, and denial rework are core model inputs.

Dual-eligible exposure is not a material launch segment for the infantile base case. Medicare eligibility under age 65 requires certain disabilities, ESRD, or ALS, while the launch population in the source pack is children younger than six years old. The base launch model therefore excludes duals as a meaningful volume bucket. If future label expansion reaches older disabled NPC patients, Medicare pays first for Medicare-covered clinician-administered services and Medicaid pays after Medicare for eligible secondary support.

The 340B operational rule is equally important: duplicate discounts are prohibited. Centers that carve Medicaid claims into 340B must maintain accurate carve-in / carve-out controls and Medicaid Exclusion File reporting. ADRABETADEX therefore needs separate provider-economics views for non-340B sites and 340B centers of excellence.

Small provider demo model

Scenario	Per administration net margin	Annualized net margin	Peak drug float	Why the result matters
Commercial medical benefit, non-340B HOPD	\$-145	\$-3,771	\$156,000	Coverage exists, but a high-cost biweekly regimen still leaves the provider slightly negative once cash timing and rework are applied.
Medicaid pediatric, non-340B HOPD	\$-2,661	\$-69,196	\$195,000	EPSDT supports access, but slower payment and NDC / denial friction make the non-340B Medicaid case visibly loss-making.
340B pediatric COE, HOPD	\$9,043	\$235,115	\$119,964	340B changes the economics immediately by lowering acquisition cost while leaving the payment side largely intact.

This demo model uses published Medicare payment benchmarks, the source-pack WAC, and fixed operational sensitivities for denial rework, receivable carrying cost, and non-reimbursed loss. The result is the central provider-access conclusion in one page: non-340B adoption is margin-tight at best, while 340B centers have a structurally stronger adoption incentive.

MLR position supported by the evidence set

- ADRABETADEX will launch into a medical-benefit market that already uses structured criteria for NPC and analogous intrathecal neurologic therapies.
- Commercial plans will manage overlap with Miplyffa, Aqneursa, and miglustat; the launch dossier therefore needs a defined regimen-positioning statement instead of generic combination-language.
- Medicaid access will be supported by EPSDT, but claim payment will depend on PAD-compliant NDC capture and state-level prior authorization execution.
- 340B is a real provider-economics accelerator and a real compliance obligation; the product will behave differently at 340B pediatric centers than at non-340B sites.
- Dual-eligible exposure is de minimis for the infantile launch population and does not change the base access model.

References

Source pack: Project Castar internal commercialization deck supplied in the source pack.

Source pack: Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for Adrabetadex supplied in the source pack.

Source pack: ADRABETADEX U.S. Reimbursement & Policy Deep Research Report supplied in the source pack.

UnitedHealthcare Miplyffa prior authorization criteria, effective April 1, 2025.

<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/h-p/PA-Notification-Miplyffa.pdf>

UnitedHealthcare Aqneursa prior authorization criteria, effective April 1, 2025.

<https://www.uhcprovider.com/content/dam/provider/docs/public/prior-auth/drugs-pharmacy/commercial/a-g/PA-Notification-Aqneursa.pdf>

HRSA 340B pricing formula page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/Pricing%20Formulas.htm>

HRSA new drug price estimation page.

<https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/New%20Drug%20Price%20Estimation.htm>

Medicaid.gov EPSDT page. <https://www.medicaid.gov/medicaid/benefits/early-and-periodic-screening-diagnostic-and-treatment>

Medicaid.gov PAD page.

<https://www.medicaid.gov/medicaid/prescription-drugs/state-prescription-drug-resources/physician-administered-drugs-pad>

Medicare and Medicaid quick guide, July 2024. <https://www.medicare.gov/publications/12207-a-quick-guide-medicare-medicaid.pdf>

CMS Medicare Part B ASP page. <https://www.cms.gov/medicare/payment/fee-for-service-providers/part-b-drugs/average-drug-sales-price>

CMS OPPS pass-through and new technology APC application page. <https://www.cms.gov/medicare/payment/prospective-payment-systems/hospital-outpatient/pass-through-payment-status-new-technology-ambulatory-payment-classification-apc>

CMS OPPS drug and biologic transitional pass-through instructions, updated March 16, 2026.
<https://www.cms.gov/files/document/determine-eligibility-drugs-biologicals-transitional-pass-through-payment-under-hospital-outpatient.pdf>

CMS HCPCS Level II coding decisions page.
<https://www.cms.gov/medicare/coding-billing/healthcare-common-procedure-system/current-prior-years-level-II-coding-decisions>

CMS 2025 first-quarter HCPCS drug and biologic coding determinations.
<https://www.cms.gov/files/document/2025-hcpcs-application-summary-quarter-1-2025-drugs-and-biologicals.pdf>